**Advanced MET exon 14 skipping lung cancer** National Cancer Control Programme (NCCP) 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 · VERSION 2

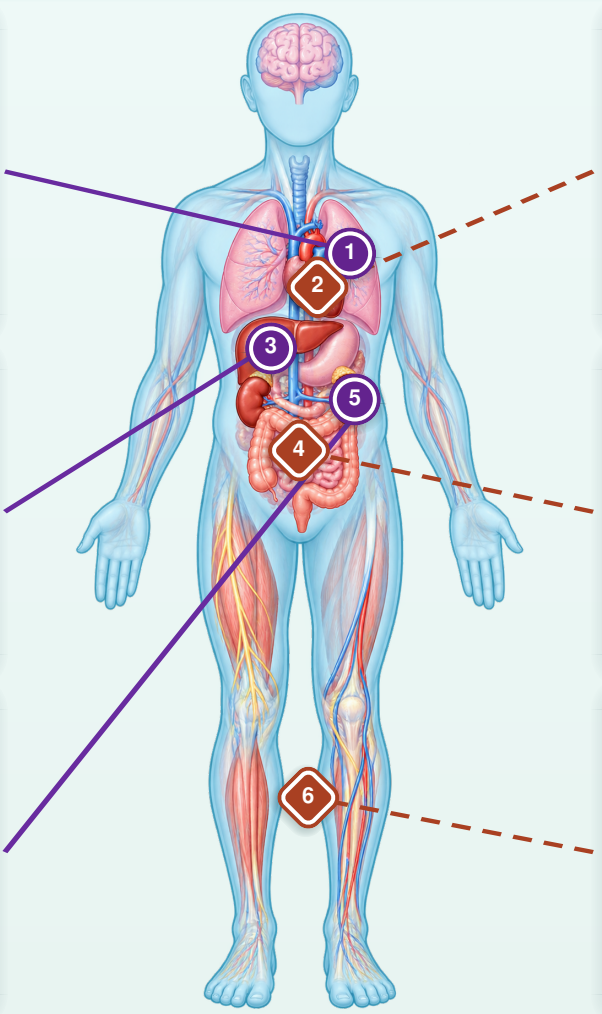
# Tepotinib monotherapy

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.




### 1 New cough or breathlessness

tepotinib

Possible lung inflammation needs urgent assessment.

### 2 Palpitations or fainting

tepotinib

Possible heart-rhythm changes need assessment.

### 3 Yellow skin or eyes

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Liver changes may be found in tests or cause symptoms.

### 4 Diarrhoea or vomiting

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Early support helps prevent dehydration.

### 5 Kidney-test changes

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Creatinine changes need assessment, including causes other than kidney injury.

### 6 Swelling and weight gain

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Fluid retention can affect movement or breathing; report changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

# Know the next step.

## Taking treatment

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Take at the same time each day with food and swallow whole. If a dose is missed, take it when remembered on the same day unless the next dose is due within 8 hours; then skip it. Do not double a dose. Ask your pharmacist if the timing is uncertain or if you vomit; do not take a replacement without advice. If swallowing is difficult, ask about the approved water-dispersion method.

## Checks and follow-up

Confirm the MET result, previous treatment, dose, other medicines and emergency contact. Check blood count, electrolytes, kidney and liver function; electrocardiogram (ECG) if clinically indicated. Discuss pregnancy, contraception and breastfeeding, and perform a pregnancy test when applicable. Repeat blood count, electrolytes, kidney and liver tests at the team's agreed reviews. ECG is used when clinically indicated. Report new lung symptoms, swelling, diarrhoea or vomiting between visits. A rise in creatinine needs assessment because the medicine can change the test result without necessarily damaging the kidneys. Scans follow the Consultant's plan.

The whole-treatment map shows the checks for this exact treatment setting.

## Support between reviews

### Swelling and weight changes

Report swelling early and discuss changes in weight, movement or breathing. The team will assess the cause and agree treatment; do not start a water tablet or change tepotinib yourself.

### Diarrhoea and fluids

Agree a diarrhoea treatment plan before starting. Begin the prescribed anti-diarrhoeal medicine at the first loose stool as instructed, and maintain fluids if able. Persistent diarrhoea, weakness or reduced urine needs prompt advice.

### Nausea medicines

Tepotinib has minimal-to-low oral emetic risk. Agree an as-needed anti-sickness plan with the team, including medicine-interaction and heart-rhythm checks; no routine infusion premedication is specified.

### Medicine review

Ask the pharmacist to check all prescribed medicines, non-prescription medicines and supplements before changes. Tepotinib can affect medicines including dabigatran, digoxin, rosuvastatin and metformin. Supportive medicines may also affect heart rhythm; individual advice is needed.

### Pregnancy, fertility and breastfeeding

Avoid pregnancy during treatment. Effective contraception is needed during treatment and for at least one week after the last dose; women using hormonal contraception should also use a barrier method. Men with a partner who could become pregnant should use barrier contraception for the same period. Do not breastfeed during treatment or for at least one week afterwards. Human fertility effects are not known; discuss fertility before starting.

**Oncology daytime:** Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.

## Your complete treatment reference

https://sactcheck.com/patient/00823/

Public information only. No personal information is encoded.